

# DexaFit Provider Marketplace: Structure, Pricing, and Provider Acquisition

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Working repo: [github.com/Tommyxu0906/dexafitEmail](https://github.com/Tommyxu0906/dexafitEmail)

## 1. What DexaFit is actually selling

The asset is not a directory. It is the ninety seconds after a client sees their body composition results and asks "okay, now what?"

That moment is rare and expensive to manufacture. A nutritionist running paid ads is buying attention from someone who might want to change their body. DexaFit can hand that same nutritionist a person who has already paid to measure their body, already sat through the results conversation, and is actively asking what to do next.

So the product is not a listing. It is **exclusive access to a qualified, data attached, high intent referral at the moment of decision**. Every decision below follows from that.

The practical consequence, and the theme of this memo: **our binding constraint is referral supply, not provider demand**. There are far more providers who want clients than there are scans producing referrals.

**Assumptions.** I did not have DexaFit's scan volume, opt in rate, or existing referral behavior, so I parameterized those. Everything else comes from public benchmarks and is named. Give me the real numbers and I will rerun the model.

## 2. Pricing: what a referral is worth

**What a provider earns from one client.** Private practice nutrition sessions run \$75 to \$175, with industry data putting average treatment value near \$155. Engagements last three to six months. Six sessions at \$150 is a lifetime value near **\$900**, with a realistic band of \$600 to \$1,500. Trainers and physicians sit elsewhere, but the method is the same.

**What they can pay to get one.** Service businesses stay healthy at a customer acquisition cost of 20 to 30 percent of lifetime value. At \$900, that is **\$180 to \$270 per acquired client**.

**What competing channels charge.**

| Channel           | Model                         | Price                         | Structural weakness                                             |
|-------------------|-------------------------------|-------------------------------|-----------------------------------------------------------------|
| Thumbtack         | Pay per lead                  | \$15 to \$80, dynamic         | Same lead sold to three to five pros. Everyone pays, one books. |
| Zocdoc            | Subscription plus per booking | \$35 to \$110 per new patient | Quoted individually, not public. Unpredictable budgeting.       |
| Opencare (dental) | Per matched patient           | \$200 to \$400                | Narrow category                                                 |

**Where a DexaFit referral lands.** It is a better unit than a pay per lead introduction on three counts. It is exclusive, routed to one provider rather than auctioned to five. It arrives with objective data, so the provider opens holding a baseline instead of asking intake questions. And the timing is precise, because the person is asking the question right now.

At a conservative one in three conversion, a provider who can spend \$180 to \$270 per acquired client can pay **\$60 to \$90 per referral** and stay well inside a healthy CAC. That number sits above Thumbtack, inside Zocdoc's band, and is defensible in a sales conversation because the provider can do the arithmetic themselves.

## 3. The model: performance first, subscription second

A \$99 per month listing is easy to justify on paper. At \$900 lifetime value a provider needs slightly more than one new client a year to break even. The arithmetic is not the obstacle. **Belief is**. No provider has seen a DexaFit referral yet, so a

monthly fee asks them to pay for volume we cannot prove. Charging subscription before we have conversion data is the most common way early marketplaces stall: supply signs up, sees nothing, and churns loudly.

**Phase 1, first 90 days.** No listing fee. \$60 per accepted referral, invoiced monthly. The provider carries zero risk and pays only for something real. DexaFit gets what it actually needs, which is conversion data by category and market.

**Phase 2, after 90 days.** Convert the cohort with their own numbers in hand. "Last quarter you accepted nine referrals and paid \$540. Featured is \$99 per month with unlimited routing and priority placement." They are not being sold a promise, they are being offered a discount on something they already buy.

Phase 1 manufactures the proof that makes Phase 2 an easy conversation. It is sales sequencing, not a pricing concession.

**The tiers we convert them to:**

| Tier            | Price                   | What it buys                                                                                                   | Who it is for                                                         |
|-----------------|-------------------------|----------------------------------------------------------------------------------------------------------------|-----------------------------------------------------------------------|
| <b>Verified</b> | Free                    | Vetted profile, appears in relevant match results, credential badge                                            | Supply base. Keeps the directory dense enough to be useful.           |
| <b>Featured</b> | \$99 / mo or \$999 / yr | Priority placement, expanded profile, booking link, direct routing of inbound referrals, performance dashboard | The core paid product. Post pilot, most revenue sits here.            |
| <b>Partner</b>  | From \$249 / mo         | Multi provider or multi location, first look routing in a defined category and market, co marketing            | Clinics and larger practices. The natural home for exclusivity deals. |

Two boundaries matter. **Free does not include routing.** Verified providers appear in results but receive no routed referrals, because the paid product is the routing, not the visibility. If free providers get referrals, nobody upgrades. **Paid placement never overrides fit.** Matching ranks on relevance first, meaning specialty, goal, geography, and credentials. Paid tiers break ties and boost position within the relevant set. They cannot surface an irrelevant provider. DexaFit's brand is why clients trust the recommendation, and a bad match damages the scan business, not just the marketplace.

## 4. Launch scope: cap the supply side deliberately

This is the point I would push hardest on.

Referral supply is set by scan volume. If we sell 200 Featured subscriptions in Boston while producing a few dozen routable referrals a month, most providers receive nothing and churn while telling other providers it did not work. Provider demand is easy to fill. **Filling it is the failure mode.**

So cap provider density per market per category, and use the cap as the pitch. "We are accepting eight metabolic health providers in Boston" is both operationally correct and the strongest line available in a cold email. The scarcity is real rather than manufactured, which is why it holds up under questioning.

Launch narrow: fat loss nutrition, metabolic health, sports performance, and post scan strength programming, in one or two markets with the highest scan volume, with a cohort of 50 to 100 vetted providers rather than 1,000.

## 5. Provider acquisition: working the 1,000 leads

**Segmentation, applied in this order.** Profession first, since the value proposition differs: nutritionists want clients who arrive with data, trainers want measurable goals and rescan driven retention, physicians and clinics want continuity of care, meaning a client they already hold data on routed back for follow up. The clinical track also uses a different call to action, since a physician referral decision is not made by booking a fifteen minute call. Then relationship temperature, warm versus cold, which changes the first line and shortens the sequence. Then market priority, since leads outside launch markets go into a waitlist sequence that also reinforces the density cap.

Sending one campaign to all 1,000 would waste the list and, more importantly, generate demand we cannot fulfill.

**Sequence: three emails, not five.** The offer is good enough that it does not need six weeks of nurture, and a short sequence lets us work the list and learn faster.

| Email | Timing | Job                                                                  |
|-------|--------|----------------------------------------------------------------------|
| E1    | Day 0  | The offer in five lines. Exclusive referrals, no fee, capped cohort. |
| E2    | Day 3  | Make it concrete. What one referral actually looks like.             |
| E3    | Day 8  | Short close with a low friction out.                                 |

**The call to action is not "pay now."** It is "apply to the founding cohort." We are collecting interest, objections, and willingness to pay, and we cannot optimize checkout for a product whose volume we have not yet proven.

**Personalization.** Merge fields drive the copy: profession, city, specialty, nearest DexaFit location, and existing relationship. Multi provider clinics and priority categories get a hand written first line. Everything else is automated. The repo implements this end to end.

## 6. Measurement

**Acquisition:** positive reply rate by segment, application rate, meetings booked, approved provider rate meaning applications that clear vetting, and paid conversion at day 90. I would not report open rate. Since mail privacy protection it mostly measures image proxy prefetching, and optimizing subject lines against it produces false winners.

**Marketplace health:** referrals routed per provider per month, provider response time to a routed referral, referral to booked appointment rate, referral to paying client rate, and client satisfaction with the match.

The single most important early number is **referrals per provider per month**. Below roughly two, the paid tier stops making sense to the provider and churn follows regardless of how good the product is.

**What the 1,000 leads are worth.** Planning assumptions, not forecasts. Every rate below is a guess until we have sent the first two hundred emails.

| Stage                                 | Conservative | Base     | Upside   |
|---------------------------------------|--------------|----------|----------|
| Total leads                           | 1,000        | 1,000    | 1,000    |
| Valid and classified                  | 850          | 850      | 850      |
| In launch markets, sent this campaign | 600          | 600      | 600      |
| Positive reply rate                   | 6%           | 12%      | 20%      |
| Interested providers                  | 36           | 72       | 120      |
| Clear vetting and fit                 | 25           | 50       | 80       |
| Paying at day 90                      | 8            | 20       | 35       |
| First cohort ARR at \$99              | ~\$9.5k      | ~\$23.8k | ~\$41.6k |

**The list alone is not a business.** Even the upside case is under \$50k ARR. These leads are worth working because they produce the first cohort and the objection data that tells us whether the model works, not because they are a revenue event.

**The base case already sits at the capacity ceiling.** Twenty paying providers each needing at least two referrals a month is forty routed referrals a month. Whether DexaFit can produce that is a function of scan volume and post scan opt in rate, not of how well the outreach performs. **The funnel tells us how many providers we can sign, and scan volume tells us how many we can keep.** If those two numbers disagree, the outreach should be throttled, not accelerated. This is the first thing I would check against real data.

## 7. Recommendation

Do not build the full marketplace before launch. Run a founding cohort of 50 to 100 vetted providers in one or two markets, priced at \$0 plus \$60 per accepted referral for 90 days, and use the outreach conversations themselves as the pricing research.

Three questions the pilot should answer before anything scales:

1. Which provider types do clients actually choose when given a real recommendation?
2. What will providers pay for, specifically: placement, exclusivity, routing, or data?
3. Can DexaFit generate enough referral volume per provider to retain a paid tier at all?

Question three decides whether this is a business or a feature. Everything in this plan is built to answer it in ninety days rather than nine months.

## Appendix. Sample email and repo

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E1 below is the cold nutritionist version. E2 and E3, the trainer and physician variants, and fifteen fully rendered examples are in the repo. Braces are merge fields.

*Subject A:* Referrals from {location} clients who already have their body comp data *Subject B:* Eight nutritionist slots in {city}

Testing specific mechanism versus scarcity.

```
Hi {first_name},

I work with DexaFit, the body composition and metabolic testing clinics. {location} runs DEXA, RMR, and VO2 max scans, and after every one of them the client asks some version of "okay, so what do I do now?"

Right now we mostly say "find a good nutritionist." We are changing that. We are building a small vetted group we route those clients to directly, with their scan data attached.

No listing fee during the pilot. You pay only when you accept a referral, and each one goes to you alone rather than to five people at once.

We are taking eight nutritionists in {city}. Worth fifteen minutes?

Tommy
```

**The repo** contains segmentation and prioritization rules, personalization field generation, templates for all three provider categories, and a sample run over fifty mock leads with output committed so it reads without running anything. I built it on mock data because I do not have the real list. That list will have a different schema and its own quality issues, but mapping it in is a short exercise and the segmentation and personalization logic carries over unchanged.

**Deliberately out of scope**, to be decided with data rather than guessed at now: provider vetting and credential verification, payment and invoicing infrastructure, the client facing matching interface, full pricing by profession since trainer and physician economics need their own derivation, and deliverability setup such as domain warming and list validation, which is execution rather than strategy.